

CFO business-case pack

Ambient Clinical Value Chain Activation

Tenant: Meridian Health System

Tenant key: meridian-health

Move ref: meridian:move:ambient-clinical-value-chain-activation

Generated: 2026-05-20

A one-document CFO pack — the seven-part answer standard: the answer, the case, the assumptions, what would make it wrong, what not to fund yet, what Tower will measure, the evidence and gaps.

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CFO business-case pack — the seven-part answer

Ambient Clinical Value Chain Activation · meridian-health. This pack answers the CFO question to the spec §9 standard: not just an answer, but the case, the assumptions, the failure modes, the funding hold, the measurement handoff and the evidence.

1 · Here is the answer

Verdict: SHAPE — do not fund as-is

The critic raised 1 blocker(s) — fund only once they are resolved. Re-shape the Move to close them.

2 · Here is the business case

Investment (effort)	\$2,481,455 – \$4,087,102 (base \$2,919,358)
Net value (post-haircut)	\$7,548,000 – \$13,209,000 (base \$10,378,500)
Net return	\$3,460,898 – \$10,727,545 (base \$7,459,142)
Payback	Not monetisable — the value rests on a baseline seed gap, so the kernel returns no payback rather than a fabricated one.
Monetisable	No
AI-build effort	\$2,136,525
Business-change effort	\$782,833 (27% of effort)

Scenario	Investment	Net return	ROI
Base	\$2,919,358	\$7,459,142	Not recorded — seed gap
Conservative	\$4,087,102	\$3,460,898	Not recorded — seed gap
Upside	\$2,481,455	\$10,727,545	Not recorded — seed gap

Downside read: The conservative case cannot be expressed in hard dollars — the value rests on a seed-gap proxy. Close the baseline gap before reading the downside as a number.

3 · Here are the assumptions driving it

The three flagged TOP MOVERS move roughly 80% of the outcome.

- [TOP MOVER] [SEED-GAP PROXY] cost_per_clinician_hour — Fully-loaded cost per clinician hour is a modeled proxy (~\$190/hr for attending physicians) until Meridian Finance attests a figure. (owner Priya Raman (VP Revenue Cycle); confidence low; impact high)
- [TOP MOVER] [SEED-GAP PROXY] locum_avoidance — Cognitive-load recovery avoids a modeled ~\$1.2M/yr of locum spend via reduced burnout-driven attrition — no locum-spend baseline is seeded. (owner Dr. Larsson (CMO); confidence low; impact high)
- [TOP MOVER] [SEED-GAP PROXY] raf_to_revenue — Each RAF point recaptured on the MA panel is worth a modeled ~\$430/yr per member; the HCC recapture range \$3.2–5.1M/yr rests on this. (owner Priya Raman (VP Revenue Cycle); confidence low; impact high)
- cdi_queue_uplift — Ambient-to-CDI integration lifts HCC-relevant query volume from ~40/wk toward the 140–180/wk pattern envelope (E25/E47). (owner Dr. Morales (CMIO); confidence medium; impact high)
- portfolio_compounding — Parallel-track execution of the three levers produces +25–35% faster outcome realisation than sequential; the low scenario assumes zero. (owner Sarah Chen (CIO); confidence medium; impact medium)
- vendor_council_commissioning — All three ambient vendor councils commission their HCC/DRG feedback loops on the G3 timeline (Abridge priority, DAX trailing 30 days). (owner Dr. Morales (CMIO); confidence medium; impact medium)
- rate_card — Blended delivery rate card reflects market rates for the integration-engineering and clinical-informatics roles in the estimate. (owner Sarah Chen (CIO); confidence medium; impact low)

4 · Here is what would make me wrong

The case breaks if the baseline seed gap is not closed — without the missing unit economics the value range is a proxy, not a forecast.

- [blocker/cfo] cfo_monetisation_blocked: Value cannot be monetised to a hard dollar figure — the gross value range rests on a seed-gap proxy. The CFO will not approve a fabricated return; the case must close the baseline gap first.
- [concern/data] data_baseline_seed_gaps: 3 baseline metric(s) are seed gaps (coverage 75%): Cost-per-clinician-hour conversion coefficient, RAF-to-revenue conversion coefficient, Locum-avoidance dollar basis. The case is grounded only on what is recorded.
- [concern/data] data_top_mover_is_proxy: 3 of the top-3 case-moving assumptions stand in for absent tenant data (cost_per_clinician_hour, locum_avoidance, raf_to_revenue). Validating these is the highest-leverage next step.

5 · Here is what I would not fund until fixed

- kill_downside_negative: Net return is negative in the conservative case (3460898.34). Kill or re-shape if the downside cannot be lifted above zero.
- kill_monetisation_unresolved: The baseline seed gap that blocks monetisation is not closed before the gate — without it the return is unverifiable.
- kill_baseline_unclosed: Baseline coverage stays below acceptable level — 3 seed gap(s) remain unresolved at the gate.
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kill_cdi_queue_undershoot: CDI-queue routing stays below 55/week at the G2 gate — the ambient-to-CDI integration, the program's core mechanism, has failed (E45).

- kill_vendor_council_blocked: HCC feedback-loop commissioning is blocked on more than one vendor council — the documentation-quality value component cannot land (E45).

Surfaced structural flags:

- [critic/blocker] Value cannot be monetised to a hard dollar figure — the gross value range rests on a seed-gap proxy. The CFO will not approve a fabricated return; the case must close the baseline gap first.
- [critic/concern] 3 baseline metric(s) are seed gaps (coverage 75%): Cost-per-clinician-hour conversion coefficient, RAF-to-revenue conversion coefficient, Locum-avoidance dollar basis. The case is grounded only on what is recorded.
- [critic/concern] 3 of the top-3 case-moving assumptions stand in for absent tenant data (cost_per_clinician_hour, locum_avoidance, raf_to_revenue). Validating these is the highest-leverage next step.

6 · Here is what Tower will measure later

3 of 4 metric(s) are wired to a recorded baseline (coverage 75%). Value loop closes: NO.

Metric	Baseline	Target	Cadence
Primary-care pajama time	129	100	monthly
CDI HCC-relevant query volume	40	160	weekly
HCC capture on the MA panel	58	63	quarterly
Cost-per-clinician-hour conversion coefficient	Not recorded — seed gap	Not recorded — seed gap	quarterly

7 · Here is the evidence I used — and what is missing

Baseline coverage 75%. 9 recorded metric(s), each with a cited source:

Metric	Value	Source	Quality
Primary-care pajama time (after-hours Epic)	129 minutes_per_day	Evidence base E17 (D06 Documentation-Time Baseline · specialty decomposition)	measured/high
MBI-HSS emotional-exhaustion burnout (elevated band)	54 percent	Evidence base E2 (D06 · MBI-HSS burnout survey wave, n=1,240)	measured/high
HCC capture rate on the MA panel	58 percent	Evidence base E28 (D10 Benchmark Comparison · cohort scorecard)	measured/medium
Note turnaround (sign-to-close)	11.4 hours	Evidence base E8 (D03 Success Metric Tree · supporting metric)	measured/medium
CDI query resolution cycle time	38 hours	Evidence base E8 (D03 Success Metric Tree · CDI cycle-time metric)	measured/medium
CDI HCC-relevant query volume	40 queries_per_week	Evidence base E25 (D08/D10 · CDI-queue routing volume benchmark)	measured/medium
Quality-measure documentation capture (HEDIS + Stars)	34 percent	Evidence base E47 (D15 Intervention Portfolio · lever outcome envelope)	measured/medium
Clinician retention (annual)	88 percent	Evidence base E9 (D16 Business Case · cognitive-load recovery component)	stated/medium
Ambulatory visits per clinician	84 visits_per_clinician_per_week	Evidence base E19 (D04 Intake · chief ambulatory officer field shadow)	stated/medium
Cost-per-clinician-hour conversion coefficient	Not recorded — seed gap	seed gap	absent/low
		seed gap	absent/low

RAF-to-revenue conversion coefficient	Not recorded — seed gap		
Locum-avoidance dollar basis	Not recorded — seed gap	seed gap	absent/low

Declared seed gaps — missing, never invented:

- Cost-per-clinician-hour conversion coefficient: Not measured. The value-at-stake model (E9) converts recovered pajama time into dollars, but the fully-loaded cost-per-clinician-hour basis is a modeled assumption — Meridian Finance has not attested a figure. Needed to monetise cognitive-load recovery.
- RAF-to-revenue conversion coefficient: Not measured. HCC recapture is sized at \$3.2–5.1M/yr (E5/E9), but the RAF-point-to-revenue coefficient on the MA book is a Revenue-Cycle estimate, not an attested baseline. Needed to monetise HCC capture lift.
- Locum-avoidance dollar basis: Not measured. The cognitive-load recovery component (E9) names locum avoidance as a value lever, but no locum-spend baseline or avoidance rate is seeded. Modeled, not measured.

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